

UK HEALTHCARE · AN EXPLAINER

From paper letters to coded records

What breaks in UK care data, and what HealthBridge does about it.

FHIR R4 | SNOMED CT | NHS number

Independent project · no patient data · not affiliated with the NHS

THE PROBLEM · 1 OF 3

Four records, four owners

GP practice

Independent contractors. The lifelong record lives in EMIS Web or SystmOne.

Hospital trust

Its own electronic patient record, separate system and login.

Pharmacy

Its own dispensing record, often unconnected to the practice.

National

NHS number, Spine and GP Connect — plumbing, not the record.

So there is no single patient record. There are several — and the thing joining them is not a database.

THE PROBLEM · 2 OF 3

The join is a letter

Hospital record
the admission and its results

→

letter / PDF
not data yet

→

Practice record
the patient history

someone must read it and code it before the record knows

Coding lag — the fact exists on paper, not in the record being searched.

Recall misses — a recall queries by code; free text is invisible.

Duplication — the same datum typed again into a second system.

THE SOLUTION · 1 OF 3

Fix the manual middle

1

Read

Letter or PDF in; diagnosis, medicine, action and dates out.

2

Propose

A SNOMED CT code per fact, with a confidence — not a decision.

3

Confirm

A clinician accepts, edits or rejects. Human, before the record.

4

File

FHIR R4 records on the NHS number, with an audit trail.

The machine does the reading. The clinician keeps the decision. Nothing is written silently.

THE SOLUTION · 2 OF 3

Built on the standards already in use

STANDARD	WHAT HEALTHBRIDGE DOES WITH IT
NHS number	Links every record to the right person, not a typed name
SNOMED CT	The proposed clinical code (SCCI0034 terminology)
dm+d	One medicine, one code, from prescribing to dispensing
FHIR R4	Records written and read back as data, not documents

Tenant isolation — one practice cannot see another's patients.

Full audit trail — who read it, who coded it, who changed it.

Interoperation — the practice keeps its own record system.

THE SOLUTION · 3 OF 3

What it will not do — and an honest limit

No diagnosis

No autonomous clinical decision, ever.

No shared pool

No cross-practice patient data.

No silent writes

Every filed fact is attributable.

No claims

No UK certification held today.

See it in 30 seconds — six clickable screens, sample data, no sign-up.

19p1362.github.io/portfolio/healthbridge

Gonela Abhilash · CPC · registered pharmacist · MSc Public Health, ARU — independent project, not affiliated with the NHS. DTAC, DSPT and DCB0129 are what a UK deployment would require first.